CGI Federal Response
to
HHS
Health Sector AI RFI

Policy References: EO 14179, M-25-21, HTI-1, TEFCA

Executive Summary

This document responds to the HHS Request for Information (RFI) on advancing AI adoption in clinical care. Our strategy aligns with the five pillars of the HHS AI Strategy: Governance and Risk Management, Infrastructure and Platforms, Workforce Development and Burden Reduction, Gold-Standard Research and Reproducibility, and Modernization of Care and Public Health Delivery.

CGI recommends targeted regulatory and policy actions to accelerate responsible AI adoption in clinical care by enabling secure, interoperable data access and aligning incentives for adoption at scale. Key priorities and expected impact are summarized below.

Regulatory priorities

- Fix data silos created by program design by enabling approved cross-program agency data sharing (real-time/streaming/bulk), supported by FHIR APIs and TEFCA alignment.
- Modernize HIPAA and ONC certification (45 CFR Part 170) to enable API-based interoperability and AI-enabled workflows (including bulk FHIR access).
- Clarify AI liability and governance (human oversight, identity/security controls) and strengthen credentialing/accreditation for safe use.
- Consider an HHS-vetted AI app store to distribute compliant tools.

Policy priorities

- Align payment and incentives to reward high-value AI that improves outcomes and reduces costs.
- Build AI readiness into programs/contracts (responsible use of AI across SDLC, operations, cybersecurity, and CX).
- Update privacy-related constraints (e.g., 42 CFR Part 2) to enable appropriate data access while maintaining protections.
- Require validation, transparency, bias monitoring, and usability supports to build trust and promote equitable access.

Expected impact

Collectively, these proposed actions would strengthen interoperability and data quality, reduce adoption risk, and support the scalable use of trusted AI to improve outcomes and operational efficiency.

AI Strategy Recommendations

To provide clear, actionable feedback to HHS, CGI organized the RFI questions into two broad categories—HHS Regulation Impact and Recommendations and HHS Policy Changes—as these categories most directly align with the underlying themes and levers for change embedded in the questions. Many of the questions focus on structural and compliance-related barriers, such as interoperability, liability, and governance, which fall under regulatory frameworks. Others emphasize incentive design, payment models, and programmatic strategies to accelerate adoption, which are inherently policy driven. By distinguishing between regulatory and policy considerations, we ensure our responses address both the foundational rules that govern AI integration, and the operational mechanisms that enable its effective use in clinical care. This approach provides clarity, avoids overlap, and highlights where HHS can exert the greatest influence to foster innovation, trust, and scalability in AI for healthcare.

HHS Regulation Impact and Recommendations

HHS regulations affect technology effectiveness, IT modernization (including AI adoption), and the ability to adapt to evolving programs and policies. Clinical care is only as effective as the governed ecosystem will allow it to be. For instance, fragmented data ecosystems, such as the lack of interoperability among EHRs, payer systems, and healthcare applications, hinder AI integration. Several regulatory complexities exist in implementation. Although FDA's Predetermined Change Control Plans (PCCP) includes some ways on how streamlined frameworks can facilitate AI innovation while ensuring safety, they are far from adequate. Addressing these challenges through unified standards and incentives can significantly enhance the integration and effectiveness of AI in clinical care. To accelerate AI adoption in clinical care, HHS should drive interagency communication to lead efforts to harmonize standards, reduce friction, and incentivize responsible use.

Data silos created by program design are a major regulatory barrier to achieving highly effective outcomes. These silos make true, near real time data sharing not only impractical, but downright impossible. Programs are designed, many by legislation and implementation interpretation, to create restrictions for data use and sharing within a narrow scope. We suggest HHS establish a mechanism to override restrictive data-use provisions for approved use cases and enable multi-mode data sharing—real-time, streaming, and bulk—across agencies such as VA and SSA. We see this as the single most transformative regulatory change that would bring true interoperability to life across HHS. Enhanced interoperability, through the adoption of FHIR-based APIs and alignment with the Trusted Exchange Framework and Common Agreement (TEFCA), would enable real-time, secure data exchange across clinical, payer, and research ecosystems, supporting population health analytics and predictive modeling.

CGI recommends modernizing HIPAA and ONC certification criteria to support API-based interoperability and AI-enabled workflows. Clarifying HIPAA's treatment of API-based exchange for AI-enabled clinical care and expanding ONC certification (45 CFR Part 170) to require standardized API access to full EHI (including bulk FHIR APIs) would help accelerate clinical decision support, care coordination, and predictive analytics while maintaining privacy and security.

Another critical area is clarifying liability and governance for AI in clinical care. Today's uncertainty around liability and indemnification for AI-informed decisions elevates risk for providers and vendors and constrains implementation. We recommend HHS develop regulatory guidance on liability allocation between clinicians and technology vendors, requiring governance models mandating human oversight in AI-assisted clinical workflows, and enforce secure management of digital identities to mitigate privacy risks. These measures will strengthen trust, accountability, and security as AI adoption accelerates. Credentialing and accreditation are also essential for safe and effective AI integration. Promoting Credentialing Service Providers (CSPs) and establishing best practices for accreditation and certification in AI-enabled clinical care would help ensure clinicians are prepared to use AI responsibly and effectively, reducing risk and improving patient outcomes.

Finally, establishing an HHS-sponsored AI app marketplace could help operationalize these recommendations by making trusted, compliant tools easier to find and adopt. A centralized, criteria-based repository would give providers a dependable source for vetted AI applications.

These recommendations ultimately depend on comprehensive, high-quality data being available through secure, interoperable mechanisms. In a recent federal healthcare engagement, we implemented an API-enabled approach to integrate data from multiple provider-domain systems into a virtualized data environment supporting advanced analytics. The same pattern can scale to clinical use cases such as predictive modeling, risk stratification, and personalized care planning. Aligning policy, interoperability, and adoption mechanisms, including a curated marketplace, positions HHS to accelerate responsible AI and improve outcomes while enhancing operational efficiency.

HHS Policy Changes

First, HHS should update payment policies to incentivize providers and payers to adopt AI-enabled tools that improve clinical outcomes and reduce costs. This can be reinforced by revisiting ONC certification criteria under 45 CFR Part 170 to require full electronic health information (EHI) access via standardized APIs, including Bulk FHIR APIs. By strengthening interoperability requirements at the policy level, HHS can expand market opportunities, support research, and accelerate development of AI-enabled clinical care tools. These changes can help AI solutions integrate more seamlessly into clinical workflows, enabling capabilities such as predictive analytics, risk stratification, and personalized care planning.

Next, HHS should broaden and standardize definitions for data use and related artifacts to improve adaptability across programs and to better accommodate rapid change. Policy updates, legislation, reauthorizations, and contracts should set clear expectations for responsible AI use and for applying AI across the SDLC, operations and maintenance, cybersecurity, and customer experience. This proactive approach will embed AI readiness into the fabric of healthcare programs, ensuring scalability and resilience.

Further, we recommend HHS make targeted updates to regulations such as 42 CFR Part 2 to improve pathways for appropriate data sharing while maintaining privacy protections for substance use disorder patient records. These updates, paired with expectations for representative datasets and continuous monitoring, would support trustworthy AI-enabled clinical decision-making and help address bias and equity concerns.

To alleviate patient and caregiver concerns, HHS can strengthen governance by requiring rigorous validation, clear human-in-the-loop oversight, and transparency for AI-assisted clinical decision-making. Policies should mandate proactive disclosure, clear communication, and access to decision summaries to build confidence in AI technologies. Improving usability through multichannel options and digital literacy coaching will help bridge the digital divide and ensure equitable access to AI-enabled care.

Looking ahead, HHS should prioritize policies that support bulk FHIR APIs to enable population health management and real-time claims analysis. These capabilities would strengthen comprehensive, data-driven care delivery while reducing administrative burden. By aligning payment incentives with economic, clinical, and social values, HHS can foster competition among AI developers and accelerate access to affordable, high-value AI tools.

Adopting these policies would enable broader, more effective use of AI in healthcare—supporting better care, more efficient operations, and continued innovation. With this foundation, HHS can lead responsible clinical AI adoption and help ensure technology delivers lasting, system-wide improvements.

Conclusion

Just as health data is often siloed by design, AI policy can become fragmented when guidance is developed program-by-program within individual agencies. To address this, we recommend an HHS-wide AI Adoption Playbook that provides a comprehensive framework for integrating AI across healthcare operations—from care delivery to backoffice functions. The Playbook should include an AI journey map that aligns people, process, and technology to HHS’s five pillars in support of One HHS, enabling a clearer path to secure, effective, and innovative AI deployments grounded in strong governance, ongoing innovation, and public trust.